

Lenacapvir for HIV in Kenya

AP Seminar

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Introduction

HIV affects 40.8 million people worldwide with 1.3 million of those cases located in Kenya (WHO 2025, NASCOP 2025). As worldwide case reports have decreased, provinces in Kenya are experiencing an uptick in HIV transmission (Kenya AIDs Progress Report 2025). Linking back to lower gender equality, higher stigmatization, and economic depravity, these trends highlight an alarming trajectory. On top of this, the 2025 cuts to USAID and the U.S. President's Emergency Plan for AIDS Relief (PEPFAR) have had a detrimental effect on Kenya's HIV prevention measures. According to research led by the Co-Director of Georgetown University's Center for Global Health Policy and Politics Sharonann Lynch in 2025, "the global HIV response is under threat like never before, with these drastic funding cuts undermining the gains of the past 25 years". It is important to note too that this recent spike has widened gender disparities as young women continue to be the most vulnerable group. However, the long term effects of these cuts may not be entirely negative. This recent uptick in transmission has spurred local government engagement in HIV prevention. Increased clinical access, decreased healthcare costs, and awareness campaigns have made an imprint in rejecting the historical attitudes towards HIV. However, stigmatization is only half of the problem. The other is advancing treatment strategies themselves. On the practical level there are two distinct pushes necessary: filling funding gaps and medical innovation. This paper will focus on the latter. A new long-lasting injectable drug, lenacapavir, was approved by the World Health Organization in 2025 and presents a revolutionary step forward in HIV medical advancement (WHO 2025). In this paper, we will begin by examining HIV in Kenya, highlighting prevention strategies, and finally evaluate how lenacapavir can fill medical gaps.

Background

Despite detecting its first case in 1984, Kenya's initial efforts were slow, characterized by denial until the late 1990s (Okuro 2022). Financial, sanitation, and healthcare barriers brought the HIV/AIDs epidemic to prevalence (Jennings et al. 2017). In 2003, the U.S. stepped in under the administration of George W. Bush, to support HIV/AIDs efforts in Kenya with the creation of the U.S. President's Emergency Plan for AIDS Relief (PEPFAR). US investment pushed the HIV rate from 7.4% to 3.2% between 2003 to 2023 (Kenya AIDs Progress Report 2025). However, recent global political shifts pose a threat. After decades of successful mitigation, HIV is rising. According to data released from the Kenya AIDS Progress Report in 2025, there was a 19% increase in HIV from 2023 to 2024: rising from 16,752 to 19,991 cases. AIDS related deaths additionally increased by 3% from 20,480 to 21,007 in 2023 and 2024, respectively. Women continue to bear a disproportionate burden, representing 66% of new infections. Urgent action is required to improve retention in care and accelerate progress toward the global target of ending AIDs as a public health threat by 2030 (Kenya AIDs Progress Report, 2025). This trajectory, taken with the abrupt foreign funding cuts in 2025, highlights the need for innovation in HIV mitigation.

Medical Prevention

Medical prevention of HIV is known as pre-exposure prophylaxis (PrEP) and, in Kenya, PrEP is available in three primary forms: daily oral pills, intravaginal rings, and long-acting injections. While daily pills and intravaginal rings are highly established methods, their real world success is often limited by the behavior burden of consistent use. Adherence to daily pills is low and declining for most participants falling from 82% to 62% between week 8 and week 52 of treatment (Kumar et al. 2025). It is for this reason that long-lasting injectables are highly

valuable. The adherence issues associated with daily oral pills and intravaginal rings are largely bypassed by the low maintenance nature of long-acting injectables. Lenacapavir, a twice-yearly injectable, is the newest advancement in this field.

Lenacapivir

A new drug twice yearly injection, Lenacapaivir, is being introduced to Kenya in 2026 and presents an exciting future. Lenacapivir's approval was preempted by two clinical trials conducted by the New England Journal of Medicine: the PURPOSE trials. PURPOSE 1, examined the protective effect of Lenacapavir in 5,338 Cisgender Women in South Africa and Uganda. Here, Bekker et al. 2024 noted that, of those on Lanacapavir, no one contracted HIV: a landmark advancement in HIV prevention showing a 100% protection rate. This high protection rate was reflected in the PURPOSE 2 trial where Kelley et al. 2025 examined 3,265 men and gender diverse people finding only 2 individuals taking Lenacapavir contracted HIV. Both Bekker at al. and Kelley et al. found contraction while on Lenacapavir was significantly less than that of the general population. In July of 2025, the World Health Organization approved the drug and in March of 2026, Kenya rolled it out. Phase one, begins by targeting 15 high-burden counties in Kenya. This will be followed by two additional phases to "progressively expand coverage nationwide, ensuring service preparedness, reliable commodity supply, and sustainable scale-up" (Ministry of Health Kenya 2026).

Financial Considerations

While Lenacapavir presents a promising future for HIV prevention, financial costs can be a barrier. South Africa, also faced with foreign funding cuts, presents an impressive case study. Pushed by funding cuts, they have localized HIV management and mitigation. By establishing domestic manufacturing through local supply chains, they have decreased reliance on global

markets and aid (Naimer 2026). In fact, South Africa is holding the first HIV vaccine clinical trial run by African scientists. South Africa is therefore dedicated to not only manufacture treatments made in the US, but actually bring drug and vaccine development to Africa (Enesco 2026). Kenya ought to follow in South Africa's footsteps by building local manufacturing lines and a scientific community working actively within the communities most affected. This localization not only works to remove reliance on foreign markets and aid, but it brings costs down for patients. As Kenya follows behind with South Africa's localization efforts, annual Lenacapavir costs are expected to be about 7,800 KES (6.60 USD) per patient, a significant reduction from approximately 42,000 USD (Ministry of Health Kenya 2026).

Another path to bringing costs down is by leveraging telehealth. Kiptinness et al. 2025 study delivering HIV prevention online instead of in-clinic. Essentially, people access virtual visits with a clinician, get approved, order an HIV self test, and then receive medication delivered to their homes. This strategy would not only help combat HIV in areas with high stigmatization by providing privacy, but would bring down the cost of treatment. Kiptinness et al. found telehealth was relatively cheap and could bring the cost of visits down to a few dollars in some cases.

Conclusion

It is important to note that even with financial feasibility, Lenacapavir alone will not independently solve the recent HIV/AIDs spike. Solvency will require local government buy-in, awareness campaigns, destigmatizations, equitable access to healthcare, sex education, and reductions to gender inequality. Without deliberate policy and financing interventions, “new prevention technologies such as Lenacapvir risk following slow adoption patterns of previous innovations, weakening a needed transformation of the HIV response” (Lynch et al. 2025). Only

when combined with these multilateral efforts, does Lenacapvir have the potential to eliminate the HIV/AIDs epidemic in Kenya. Taken together, the 2025 foreign funding cuts, if handled correctly, may actually push Kenya to localize efforts, decrease foreign dependency, and ultimately stop the spread of HIV.

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